



Carisurg

Assignment 4

Report

Focus Topic: SpO2 —Oxygen Saturation.

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Oxygen Saturation (SpO₂)

Having worked with FiO₂ — the fraction of inspired oxygen — in our dataset, it became apparent that its most clinically meaningful partner was absent. FiO₂ tells you what oxygen concentration a patient is receiving, but without SpO₂, you have no way of knowing how effectively their body is actually using it. It felt only natural to write about SpO₂ as the missing half of that picture.

Oxygen Saturation (SpO₂) is a measure of the percentage of haemoglobin in the blood that is carrying oxygen, typically measured at triage using a pulse oximeter placed on the fingertip. It represents how effectively the lungs are transferring oxygen into the bloodstream and is expressed as a percentage.

The normal clinical range for SpO₂ in a healthy adult is **95–100%**.

A reading **below 95%** demands clinical attention, and a reading **below 90%** is considered **hypoxaemia** — a state of dangerously low blood oxygen that requires immediate intervention.

In patients with chronic respiratory conditions such as COPD—Chronic Obstructive Pulmonary Disease, a baseline SpO₂ of **88–92%** may be their normal, which is an important nuance for triage nurses to account for when interpreting readings.

In an emergency department triage setting, SpO₂ is one of the fastest and least invasive vital signs to obtain. A drop in SpO₂ can precede visible signs of respiratory distress, making it an early warning indicator that a patient is deteriorating before they appear overtly unwell.

When read alongside FiO₂, the picture becomes even more powerful — a patient on high-flow oxygen (high FiO₂) with a low SpO₂ is in far more danger than their oxygen delivery alone would suggest, as it indicates their lungs are failing to utilise the oxygen being provided.

At Mercer General, capturing SpO₂ routinely alongside FiO₂ would significantly strengthen the dataset's ability to identify patients at respiratory risk early in the triage pathway.